

Interview Transcript with Dr. Erik Vogel (Berlin, Germany) Epidemiologist & Policy Advisor

Interviewer: Dr. Vogel, thank you for taking the time to speak with us. Could you start by telling us about your background and your involvement during the pandemic? Dr. Vogel: Certainly. I'm Dr. Erik Vogel, an epidemiologist by training and a senior advisor to the Robert Koch Institute (RKI) here in Berlin. Before COVID-19, my work primarily focused on influenza surveillance and modeling infectious disease dynamics for public health policy.

When SARS-CoV-2 appeared, my team's role expanded overnight. We were responsible for creating real-time models of transmission, hospital capacity forecasts, and advising the federal government on mitigation measures. It was the most intense, complex, and emotionally charged work of my career.

Interviewer: Epidemiology suddenly became a household term in 2020. How did it feel to see your field at the center of national conversation? Dr. Vogel: Surreal. Epidemiologists are used to working quietly behind the scenes—our graphs rarely make the evening news. But suddenly, everyone was reading R-values and incidence curves over breakfast.

It was gratifying to see data valued, but also daunting. Numbers without context can mislead. A case curve doesn't tell you about testing bias or demographic spread. One of our constant battles was against oversimplification—by both the media and, at times, policymakers.

Interviewer: Can you describe the initial modeling challenges in early 2020? Dr. Vogel: The hardest part was data scarcity. Early models had to rely on assumptions from China and Italy—vastly different healthcare systems and demographics. We were modeling with incomplete information.

By March 2020, Germany had multiple models predicting hospital overload, but with wide uncertainty ranges. Some officials found the uncertainty frustrating—they wanted precise forecasts. We had to explain that models aren't crystal balls; they're maps with fog.

Another challenge was behavioral adaptation—once people change behavior, the model's predictions change too. Epidemiology is as much about sociology as it is about biology.

Interviewer: How did Germany's policy apparatus engage with scientific recommendations? Dr. Vogel: Initially, quite well. The RKI had direct channels to the Chancellor's office. Decisions like the first lockdown were made with heavy reliance on epidemiological data.

However, as the crisis evolved, tension grew between scientific timelines and political timelines. Science demands caution; politics demands decisiveness. Our models often came with caveats—if mobility drops by X percent, then Y may happen—but politicians needed actionable sentences, not conditional paragraphs.

To their credit, most policymakers listened. But we learned that evidence must be paired with

empathy people don't follow data; they follow trust.

Interviewer: What were the most critical data-driven decisions taken early on? Dr. Vogel: The nationwide school closures in March 2020 were pivotal. Our contact tracing data showed that asymptomatic transmission among households and communities was driving spread, and children played a role, though modestly.

Another key decision was the staggered reopening strategy. We proposed reopening sectors based on contact density and ventilation feasibility. For example, outdoor retail before indoor dining. That framework became part of Germany's lockdown exit matrix.

Finally, the vaccination prioritization model we used age-structured population data and comorbidity risk to optimize early dose allocation. That saved thousands of lives.

Interviewer: Germany's data transparency was often praised internationally. How was that achieved? Dr. Vogel: Through the Corona-Datenspende app and RKI's open dashboards. We believed

that citizens should see the same data we used. Transparency fosters accountability.

However, data integration wasn't easy. Each federal state had its own reporting systems. Harmonizing that into a unified national dataset required digital modernization that Germany had long postponed. COVID forced us to build a 10-year infrastructure in 10 months.

We now have the SurvNet 3.0 system a real-time case and hospital surveillance platform. It's one of the quiet successes of the pandemic.

Interviewer: What was your view on public perception and misinformation? Dr. Vogel: That was perhaps the most frustrating part. We faced an infodemic an overabundance of false or distorted information. Epidemiologists became targets of online abuse. Colleagues received threats simply for presenting data.

At first, we responded with more charts, thinking facts would fix fear. They didn't. We realized that emotion trumps logic. That's when we began collaborating with psychologists and communication experts.

We learned to speak not just in numbers, but in narratives. Flattening the curve was not just a model, it was a story. That phrase probably saved more lives than any individual policy.

Interviewer: How did Germany balance federal and state-level responses? Dr. Vogel: Germany's federal structure is both a strength and a challenge. Decentralization allows flexibility, but coordination takes effort. During peak waves, we had nightly inter-state meetings epidemiologists, health ministers, data analysts to synchronize thresholds for action.

Sometimes, states acted faster or slower depending on political pressure. But overall, the collaborative culture held. Even when debates grew heated, we shared one objective: avoid collapse of the healthcare system.

The pandemic showed that data federalism can work but only when underpinned by shared transparency and mutual respect.

Interviewer: What role did vaccination campaigns play from an epidemiological standpoint? Dr.

Vogel: They changed everything. Once vaccine coverage reached 60%, the epidemic curve decoupled

from hospitalizations. We could predict that shift mathematically, and seeing it happen was remarkable.

We also modeled breakthrough infections and waning immunity, which guided booster recommendations. But public communication was tough explaining that vaccines prevent severity, not infection, led to confusion.

Still, the data was clear: vaccination turned COVID from a mass-casualty threat into a manageable disease. That's an extraordinary scientific achievement.

Interviewer: Were there moments of disagreement between scientists and policymakers? Dr. Vogel:

Of course. One notable debate was around school reopening during the third wave.

Epidemiologically, we recommended a phased approach with ventilation standards. Some political leaders wanted full reopening sooner due to public pressure.

Ultimately, compromise prevailed. In science-policy interface, disagreement is natural the key is maintaining respect. The danger is not disagreement, but disconnection.

Interviewer: How do you reflect on Germany's handling of the pandemic overall? Dr. Vogel: I would say Germany managed competently but cautiously. Our mortality rates were lower than many European peers, thanks to strong early testing, robust ICU capacity, and disciplined citizens.

However, we struggled with bureaucratic slowness particularly in digital contact tracing and vaccination logistics early on. The lesson is clear: in a crisis, agility is as important as accuracy.

On balance, I believe our response demonstrated that democracy can act decisively without abandoning deliberation that's no small feat.

Interviewer: What do you consider the biggest lesson for future pandemics? Dr. Vogel: Three things: 1. Preparedness is perishable it must be practiced, not just planned. 2. Data must flow at the speed of decisions. Real-time epidemiology saves lives. 3. Communication is part of containment. Without trust, even the best data fails.

We need permanent data infrastructures, not temporary dashboards. And we need scientists trained to communicate uncertainty gracefully that's the next frontier.

Interviewer: Personally, what has this experience taught you? Dr. Vogel: That humility is the foundation of science. No model, no dataset, no forecast can capture human behavior completely. But that shouldn't paralyze us it should make us collaborate better.

It also taught me the power of empathy in policy. Behind every case is a person with a story. Epidemiology without humanity is just arithmetic.

I hope future scientists remember that our equations describe lives not just numbers.

Interviewer: Beautifully said. Thank you for sharing your experience, Dr. Vogel. Dr. Vogel:

Thank you. If the data of this era helps us act faster and kinder next time, then the work was worth it.

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